Pre-IVF Diagnostic Summary for 2026

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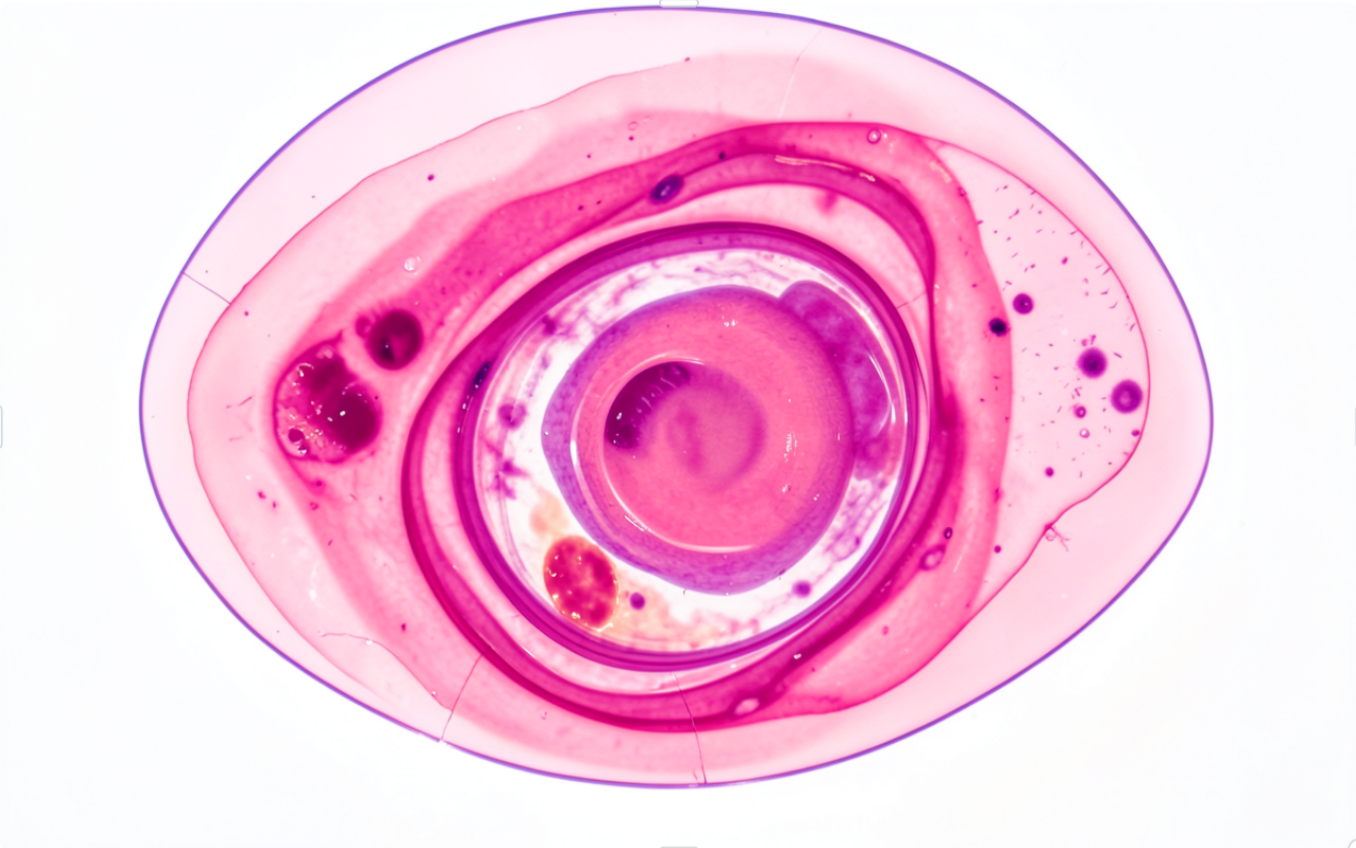


2026 IVF Pre-Treatment Assessment Report

1.0 Executive Summary

This report details the mandatory baseline diagnostic assessments, including pelvic ultrasound and endocrine profiling, required to establish a patient's reproductive health profile before initiating ovarian stimulation for IVF. The findings confirm a favorable baseline for proceeding with a controlled ovarian stimulation protocol in the 2026 treatment cycle.

2.0 Introduction

The purpose of this document is to formally record the results of pre-treatment pelvic ultrasounds and hormonal screenings. This data is critical for customizing the IVF stimulation protocol to the patient's specific physiological markers, thereby optimizing the potential for successful oocyte retrieval and subsequent embryo development.

3.0 Objectives

- To assess ovarian reserve via Antral Follicle Count (AFC) and serum Anti-Müllerian Hormone (AMH).
- To evaluate uterine receptivity and cavity integrity, measuring endometrial lining thickness and checking for structural anomalies.
- To identify any anatomical abnormalities (e.g., polyps, fibroids, cysts) that could impact treatment.

4.0 Scope & Limitations

This analysis focuses strictly on the pre-cycle diagnostic phase. It does not cover the pharmacological stimulation, oocyte retrieval, fertilization, embryo culture, or embryo transfer procedures. The findings are based on a single-point assessment and do not predict dynamic responses to medication.

5.0 Methodology

The evaluation was conducted through a comprehensive review of transvaginal ultrasound (TVS) imaging and baseline endocrine profiles. The TVS was performed on cycle day 2-3 to assess the AFC and endometrial status. Blood work analyzed Follicle-Stimulating Hormone (FSH), Estradiol (E2), and AMH levels.

6.0 Data Sources

- Clinical transvaginal ultrasound records and imaging.
- Laboratory assays for FSH, AMH, and E2.
- Patient-provided medical and gynecological history.

7.0 Key Findings Overview

Scanning confirms an adequate Antral Follicle Count (AFC) bilaterally and a healthy, trilaminar endometrial lining of optimal thickness for the early follicular phase. Hormonal markers align with expected ranges for the patient's clinical age group.

8.0 Detailed Findings

8.1 Uterine Assessment

The uterine cavity is clear with no evidence of polyps, submucosal fibroids, or synechiae. Endometrial lining measured at 4.2 mm, displaying a characteristic trilaminar pattern.

8.2 Ovarian Reserve & Morphology

Ovarian reserve markers are within the expected range for the 2026 clinical age brackets. A detailed breakdown is provided below.

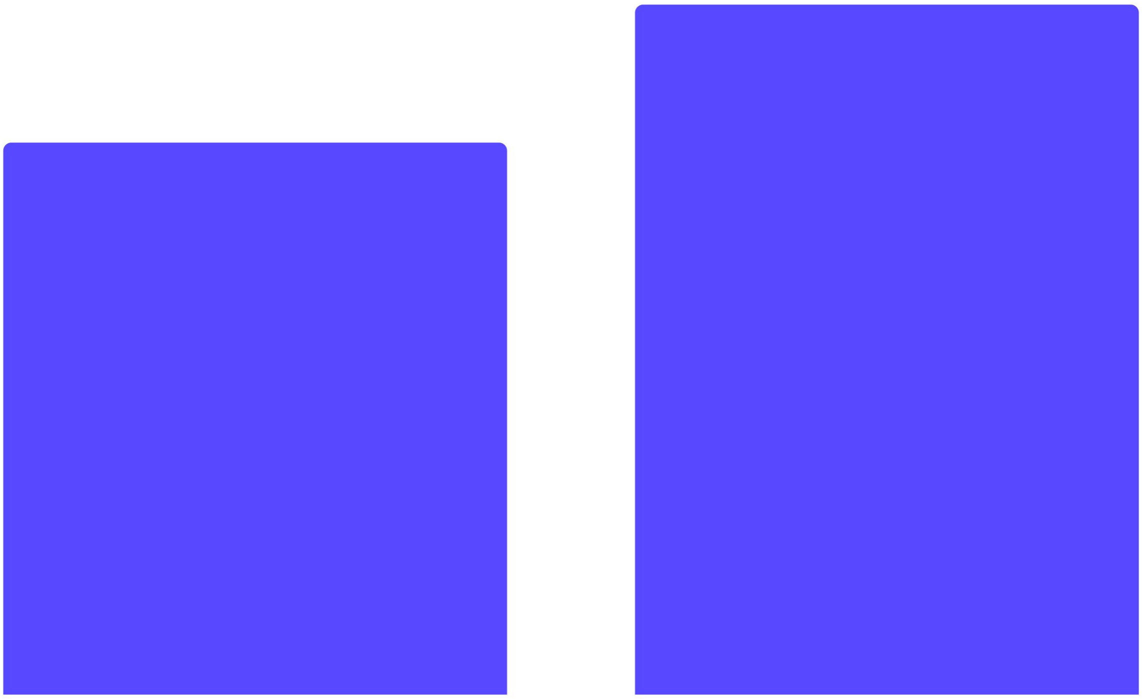
Parameter	Left Ovary	Right Ovary	Reference Range
Antral Follicle Count (AFC)	8	10	≥ 6 total
Dominant Follicle Size	None	None	N/A (Baseline)
Notable Morphology	Simple cyst (12mm)	Unremarkable	N/A

8.3 Hormonal Profile

Hormone	Value	Units	Interpretation
Follicle-Stimulating Hormone (FSH)	6.8	IU/L	Normal
Anti-Müllerian Hormone (AMH)	2.4	ng/mL	Normal

Estradiol (E2)	42	pg/mL	Normal (Baseline)

9.0 Data Visualization



10.0 Analysis & Interpretation

Current scanning and laboratory data suggest a high probability of successful multi-follicle recruitment during the stimulation phase. The total AFC of 18 and normal AMH level indicate a satisfactory ovarian reserve. The thin, trilaminar endometrium is an ideal baseline.

11.0 Risks & Issues

A potential issue identified is a simple cyst (12mm) in the left ovary. While likely non-functional and common, it may require monitoring during early stimulation to ensure it does not interfere with follicular growth or medication response.

12.0 Recommendations

Based on the favorable diagnostic profile, it is recommended to proceed with the prescribed antagonist stimulation protocol, commencing gonadotropin injections on Day 2 of the upcoming menstrual cycle.

13.0 Action Plan / Next Steps

1. Schedule a suppression check (ultrasound and blood work) for Day 2 of menses.
2. Begin the prescribed gonadotropin injection regimen following the suppression check confirmation.
3. Monitor the simple cyst in the left ovary during the first stimulation scan.

14.0 Impact & Outcomes

Optimized treatment planning based on this accurate baseline scanning and profiling is projected to improve cycle efficiency, oocyte yield, and ultimately, cumulative live birth rates for the 2026 treatment cohort.

15.0 Conclusion

The patient is cleared for IVF treatment following a satisfactory pre-treatment medical and scanning evaluation. No significant contraindications or anatomical barriers to treatment were identified.

16.0 References / Citations

- Standard Fertility Association Guidelines for Pre-IVF Assessment, 2026 Edition.
- Clinical Imaging Protocols for Reproductive Medicine, Clinic Internal Document v3.1.

17.0 Appendix

Detailed ultrasound imagery and complete hormone level worksheets are attached to the electronic medical record under the patient's file and this report's unique identifier.

18.0 Acknowledgements

Credits to the Reproductive Endocrinology Department for clinical oversight and the Diagnostic Imaging Center for precise scanning and analysis.

19.0 Thank You Page

We appreciate your trust in our clinical diagnostic services and are committed to supporting your treatment journey with data-driven care.